

ZOLTRIM 480 PREMIX

FOR ANIMAL USE ONLY

Brand or Product Name :

Zoltrim 480 Premix

Name and Strength of Active Substance(s)

Each kg contains 400 g Sulfamethoxazole and 80 g Trimethoprim.

Product Description

Zoltrim 480 Premix is a white or almost white powder containing 400g/kg Sulfamethoxazole and 80g/kg of trimethoprim.

Pharmacodynamics

Alone, sulfamethoxazole is bacteriostatic agent and trimethoprim is bactericidal, but in combination, the potentiated sulfamethoxazole is bactericidal. Potentiated sulfamethoxazole sequentially inhibit enzymes in the folic acid pathway, thereby inhibiting bacterial thymidine synthesis. Sulfamethoxazole blocks the conversion of para-aminobenzoic acid (PABA) to dihydrofolic acid (DFA), and trimethoprim blocks the conversion of DFA to tetrahydrofolic acid by inhibiting dihydrofolate reductase. Antimicrobial activity : Both Sulfamethoxazole and trimethoprim are broad-spectrum antimicrobial agents. Their combination offer synergism against many Gram positive and Gram negative bacteria as well as broader spectrum of activity. Gram-positive bacteria that are generally susceptible include most streptococci, many strains of staphylococci and nocardia. Many Gram negative organisms of the family Enterobacteriaceae are also susceptible. Some protozoa (pneumocystis carinii, Coccidia and Toxoplasma) are also inhibited by the combination. Resistance will develop slower to the combination of drugs than to either one alone.

Pharmacokinetics

Trimethoprim/sulfa is well absorbed following oral administration, with peak levels occurring about 1-4 hours after dosing. Trimethoprim/sulfa is well distributed in the body. When meninges are inflamed, the drugs enter the CSF in levels of about 50% of those found in the serum. Both drugs cross the placenta and are distributed into milk. Trimethoprim/sulfa is both renally excreted unchanged via glomerular filtration and tubular secretion and metabolized by the liver. The sulfas are primarily acetylated and conjugated with glucuronic acid and trimethoprim is metabolized to oxide and hydroxylated metabolites.

Indication

Pig (Less than 4 months old) : Bacterial diarrhea caused by E. coli, Swine pleuropneumonia

Chicken (Except laying hens) : coccidiosis, colibacillosis

Recommended Dosage

For oral administration after incorporation into complete feed.

Pig (Less than 4 month old)

Bacterial diarrhea caused by Escherichia coli : Mix 420 g to 830 g per ton of feed and administer for 5 days.

Swine pleuropneumonia : Mix 170 – 420 g per ton of feed and administer for 7day.

Chicken (Broilers)

Coccidiosis : Mix 420 g to 830 g per ton of feed and administer for 3 - 5 days.

Colibacillosis : Mix 420 g to 830 g per ton of feed and administer for 5 day.

Mixing Directions :

To ensure thorough dispersion of the product it should be first mixed with 20kg of feed ingredients before incorporation into final feed.

Mode of Administration :

To be mixed thoroughly with animal feed.

Contraindications

Do not use in animals suffering from severe liver or kidney disease, oliguria or anuria.

Do not use in animals with impaired haematopoietic systems.

Do not use in case of known hypersensitivity to sulfonamides or trimethoprim.

Warnings and Precautions

This combination should be used with caution in animals with pre-existing hepatic disease. Care should be taken to avoid accidental ingestion. In case of accidental ingestion, wash out mouth immediately with water and seek medical advice. May cause irritation or sensitization by skin contact. Avoid contact with skin and eyes. In case of accidental spillage onto the skin or eyes, rinse the affected area with clean running water. Seek medical advice if irritation persists. People with a known sensitivity to the ingredients in this product should avoid using the product. Wear overalls and impervious gloves when mixing and handling the product. Open bottle with care to avoid spillage. Use safety goggles to avoid contact with eyes. Wash hands and exposed skin after handling the product. Do not eat, drink or smoke when handling this product.

Respiratory Protection	: Not require
Hands Protection	: Wear impervious gloves when handling liquid.
Eye Protection	: Wear safety goggles to avoid contact with eyes.
Skin Protection	: Wear protective clothing to minimize skin exposure

Interactions with Other Medicaments

Trimethopim/sulfa may prolong the clotting times in patients receiving coumarin(warfarin) anticoagulants. Sulfonamides may displace other highly bound drugs, such as methotrexate, phenylbutazone, thiazide diuretics, salicylates, probenecid and phenytoin. Although the clinical significance of these interactions is not entirely clear, patients should be monitored for enhanced effects of the displaced agents. Antacids may decrease the bioavailability of sulfonamides if administered concurrently. Trimethoprim may decrease the therapeutic effect of cyclosporine (systemic) and increase the risk of nephrotoxicity developing.

Pregnancy and lactation

Safety of trimethoprim/sulfa has not been clearly established in pregnant animals. Use during pregnancy only if risks outweigh the benefits since folic acid metabolism may be affected. Safety of trimethoprim/sulfa has not been clearly established in lactating animals.

Side Effects

Trimethoprim has shown teratogenic effects in laboratory animals. Sulfamethoxazole, was documented to cause growth inhibition of bone marrow cells in experimental animals and should be monitored carefully when using and discontinue use if abnormalities are observed. A diminished water intake in chickens may occur occasionally. Hypersensitivity reactions can occur rarely.

Overdose and Treatment

Manifestations of an acute overdosage can include symptoms of GI distress (nausea, vomiting, diarrhea), CNS toxicity (depression, headache and confusion), facial swelling, bone marrow suppression and increase serum aminotransferases. Oral overdoses can be treated by emptying the stomach and initiating symptomatic and supportive therapy. Acidification of the urine may increases the renal elimination of trimethoprim, but could also cause sulfonamide crystalluria. Complete blood counts and other laboratory parameters should be monitored as necessary. Bone marrow suppression associated with chronic overdoses may be treated with folic acid (leucovorin) if severe. Peritoneal dialysis is no effective in removing TMP or sulfas from the circulation.

Storage Conditions

Store airtight and below 30°C. Keep out of reach of children. Jauhkan ubat dari kanak-kanak.

Shelf-life

As packaged for sale: 3 years from the date of manufacture.

After first opening of the immediate packaging: 24 hours

After reconstitution: 24 hours

Withdrawal Period(s)

Poultry & Pigs : 7 days. Not authorized for use in laying birds producing eggs for human consumption.

Maximum Residual Limit (MRL)

All food-producing species : Sulfa : Milk (cattle) = 0.01mg/kg, Edible offal, muscle (chicken and cattle) = 0.10mg/kg

Trimethoprim : Milk (cattle) = 0.05mg/kg, Edible offal, muscle (chicken and mammalian) = 0.05mg/kg, egg(chicken) = 0.05mg/kg [Ref. REGOVP, Version 2, dated Aug 2008]

Disposal of Containers

Emptied bag must be disposed safely according to local government regulations.

Dosage Forms and packaging available

25 kg in PP-lined paper bag.

Name and Address of manufacturer

Life Biopharma Sdn. Bhd.

No. 250-251, Jalan Industri Galla 13, Kawasan Perindustrian Galla, 70200 Seremban, Negeri Sembilan, Malaysia.

Date of Revision of Package Insert

2014-12-02